Safety



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

This ATI Engage material is the sole and exclusive copyrighted property of Assessment Technologies Institute, L.L.C. (“ATI”) and is provided under license to ATI’s clients, solely for use by each such client’s nursing program utilizing ATI Engage resources under license by ATI. As a client of ATI, you are permitted to post this material on your institution’s LMS for the duration of your institution’s license to use of ATI’s Engage with its faculty and students. You must ensure that all copyright notices and other proprietary ATI marks herein legibly appear when posted to your LMS.

This is provided under license to your institution and not to you, personally, so you may not use this for any purpose outside of your responsibilities with the institution to whom the license was granted by ATI. If your institution terminates its use of ATI Engage, your right to use this material immediately ceases and you will promptly remove this material from your institution’s LMS and make no further use of this material for any purpose.

You must not let any third party outside your institution access or use this material. You agree not to post this material on any social media platform. Your right to use and post this material on your institution’s LMS is subject to this notice.



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

The Joint Commission (TJC)

- Established in 1951.
- Impartial national organization
  - accredits hospitals and other health care facilities based on their safety performance, policy, procedures, practice, and outcomes.
- More than 22,000 health care institutions and programs are currently accredited by TJC.
- TJC facility evaluation performance scores include
  - monthly submission of the hospital's safety and quality performance outcomes to an independent vendor that quantifies
  - identifies safety trends, both positive and negative, four times a year.
- These quarterly performance measures can be tracked as well as the action plans specific for that outcome.



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Established in 1951.
Evolved as an impartial national organization that accredits hospitals and other health care facilities based on their safety performance, policy, procedures, practice, and outcomes.
More than 22,000 health care institutions and programs are currently accredited by TJC.
TJC facility evaluation performance scores include the monthly submission of the hospital's safety and quality performance outcomes to an independent vendor that quantifies and identifies safety trends, both positive and negative, four times a year.
These quarterly performance measures can be tracked as well as the action plans specific for that outcome.

National Patient Safety Goals (NPSG) and Standards of Compliance

- Initiated in 2002 by TJC.
- Identify established relevant safety practices health care institutions should accomplish.
- Since most health care organizations embrace and routinely practice these goals, they are cataloged into a register of adopted "Standards of Compliance" that must be met on a consistent basis.
- There are now over 250 standards of compliance.



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

National Safety Goals

- In contrast, newly created and approved National Safety Goals are endorsed every year based on the adverse events and sentinel events that have trended nationally in health care facilities.
- Current goals:
  - Identify client correctly
  - Improve staff communication
  - Use medications safely
  - Use alarms safely
  - Prevent hospital acquired infections
  - Identify client safety risks: Reduce the risk of suicide
  - Universal protocol: Prevention of adverse events in surgery



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

The goal of identifying clients accurately has two components: use two client identifiers and ensure that the medication, treatment, procedure, or care is intended for that specific client. Client identifiers may include the client's name, date of birth, designated hospital number, telephone number, or alternative client-specific documentation

The goal of improving staff communication refers to reporting critical results promptly. Critical results may be defined as laboratory or diagnostic procedure results that are outside the expected range and which can be life-threatening or potentially fatal if not immediately improved

Meds - This is a three-pronged goal: label all medications, especially before a procedure; decrease the possibility of medication errors with anticoagulant (blood-thinning) medications; and establish an accurate and current medications list for the client and have the provider reconcile the list with the new drugs ordered.

To reduce the risk of harm to clients, NPSG promotes using clinical alarm

systemsin health care settings. These audible alert devices, which are built into medical equipment, are intended to warn health care providers that a potentially serious event may be occurring. Clinical alarm systems also indicate when a machine malfunctions.

Nosocomial infections, also known as hospital–associated infections (HAIs), are infections that are acquired during clients' hospital stay. The *Healthy People 2030*initiative reports that more than 500,000 persons acquire HAIs in health care facilities each year. These infections arise in clients who are receiving care for other clinical conditions.

Death by suicide is the tenth leading cause of death in the United States as a whole and the second leading cause of death among persons between ages 10 and 34 in this country. This NPSG requires screening of clients who may be at risk for suicide or who have intentions of suicide. Clients who are at risk for suicide, as determined by the provider, are placed on round-the-clock surveillance while in a nonpsychiatric hospital. Potentially harmful items should be removed from the room by nursing staff. Using an environmental checklist will help nursing staff remove items that may pose a risk for clients at risk for suicide. Searching and limiting items brought into the client's room and managing visitation by nonhospital staff are also necessary.

Preventing surgical adverse events is a critical factor in TJC's NPSGs. An adverse event is a situation or circumstance that causes unexpected harm to the client. TJC mandates that a time-out—that is, a pause in all personnel activities within the operating or procedure room—take place before each surgery. This brief suspension allows for the confirmation of the correct client, site, and procedure to be performed.

Unexpected Events Including Sentinel Events

- Near miss
- Patient safety event
- Sentinel event (never event)



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

A near miss is a potential error or event or circumstance that could have caused harm but was caught and avoided.
A patient safety event is an unexpected event or circumstance that occurred without injury to the patient.
A sentinel event (never event) is a critical, unexpected adverse event that caused severe physical or psychological harm to a patient, including death and dismemberment; permanent injury; or severe, temporary injury.

1 out of 10 causes of death and disability in the world are due to adverse events from unsafe care
Medication errors are a leading cause of injury and avoidable harm
HAI's occur in 7 out of every 100 hospitalized patients

Never events should never happen

Centers for Medicare and Medicaid now deny hospitals higher payments if there is a never event or complication from a never event.

Many hospital-acquired conditions are nurse sensitive indicators

Never Events



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Surgical or invasive procedural events

•Surgery or other invasive procedure performed on the wrong site
•Surgery or other invasive procedure performed on the wrong patient
•Wrong surgical or other invasive procedure performed on a patient
•Unintended retention of a foreign object in a patient after surgery or other invasive procedure
•Intraoperative or immediately postoperative/post-procedure death in an American Society of Anesthesiologists Class I patient

Product or device events
•Patient death or serious injury associated with the use of contaminated drugs, devices, or biologics provided by the healthcare setting
•Patient death or serious injury associated with the use or function of a device in patient care, in which the device is used for functions other than as intended
•Patient death or serious injury associated with intravascular air embolism that occurs while being cared for in a healthcare setting

Patient protection events
•Discharge or release of a patient/resident of any age, who is unable to make decisions, to other than an authorized person
•Patient death or serious injury associated with patient elopement (disappearance)
•Patient suicide, attempted suicide, or self-harm that results in serious injury, while being cared for in a healthcare setting

Care management events
•Patient death or serious injury associated with a medication error (e.g., errors involving the wrong drug, wrong dose, wrong patient, wrong time, wrong rate, wrong preparation, or wrong route of administration)
•Patient death or serious injury associated with unsafe administration of blood products
•Maternal death or serious injury associated with labor or delivery in a low-risk pregnancy while being cared for in a healthcare setting
•Death or serious injury of a neonate associated with labor or delivery in a low-risk pregnancy
•Patient death or serious injury associated with a fall while being cared for in a healthcare setting
•Any stage 3, stage 4, and unstageable pressure ulcers acquired after admission/presentation to a healthcare setting
•Artificial insemination with the wrong donor sperm or wrong egg
•Patient death or serious injury resulting from the irretrievable loss of an irreplaceable biological specimen
•Patient death or serious injury resulting from failure to follow up or communicate laboratory, pathology, or radiology test results

Environmental events
•Patient or staff death or serious disability associated with an electric shock in the course of a patient care process in a healthcare setting
•Any incident in which a line designated for oxygen or other gas to be delivered to a patient contains no gas, the wrong gas, or is contaminated by toxic substances
•Patient or staff death or serious injury associated with a burn incurred from any source in the course of a patient care process in a healthcare setting
•Patient death or serious injury associated with the use of physical restraints or bedrails while being cared for in a healthcare setting

Radiologic events
•Death or serious injury of a patient or staff associated with introduction of a metallic object into the MRI area

Potential criminal events

•Any instance of care ordered by or provided by someone impersonating a physician, nurse, pharmacist, or other licensed health care provider
•Abduction of a patient/resident of any age
•Sexual abuse/assault on a patient or staff member within or on the grounds of a healthcare setting
•Death or serious injury of a patient or staff member resulting from a physical assault (i.e., battery) that occurs within or on the grounds of a healthcare setting

Responsibility

- Administration, risk management, and nurse leaders should advocate that all team members report unexpected events and near misses.
- Why do we think this does not happen?



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Barriers to Event and Near Miss Reporting

- Fear of repercussions or backlash to the reporting individual or the team.
- Lack of time to write the report.
- Unclear facility policies and standards.
- Bullying.
- Insufficient education and training.
- Lack of understanding of the roles and responsibilities of team members.
- Favoritism and influence of some employees.



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Home Safety

- Educate clients on safety when returning home from hospital
- Consider diagnosis and any procedures/medications from admission
- Adequate lighting?
- Handle bars or chair in shower?
- Throw rugs on floor?



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

home safety – beyond the home

- Rural setting
- Transportation
- Communication connectivity
- Caregiver capacity



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Rural setting: Distance from emergency services, nearest ER, or urgent care. If a patient lives 45+ minutes from a hospital, what does that change about discharge instructions and when to call 911? Who do they call first — the provider, 911, a neighbor?

Transportation: Does the patient have reliable transportation to follow-up appointments? Can they get to a pharmacy? For patients on Medicare in rural areas, medical transport may be a covered benefit — does the nurse know to ask about this?

Communication and connectivity: Does the patient have a phone? Reliable cell service? This matters for telehealth follow-up and for calling with questions post-discharge.

Caregiver capacity: Who is actually at home? What are their physical and cognitive limitations? What happens if that caregiver is also ill?

Interprofessional Collaboration

- Bedside nurse
  - Initiate referrals early
  - Communicate client situation
- Case manager
  - Coordinates discharge plan
  - Identifies resources and arranges resources
    - Home health
    - Durable medical equipment
- Social work
  - Address social determinants of health
    - Housing stability, financial barriers, caregiver strain, food security
- Home health nurse
  - Continues skilled nursing care after discharge



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Fall Risk and Prevention

- Complete Morse Fall Scale each shift
- Keep environment free of clutter
  - Place bedside commode out of sight
- Ensure call light within reach and educate
- Place yellow wrist band and yellow non-skid socks
- Hang sign per hospital policy
- Leave room with bed low and locked
- Ensure adequate lighting
- Consider family involvement



Morse Fall Scale

Fall Risk is based upon Fall Risk Factors and it is more than a Total Score. Determine Fall Risk Factors and Target Interventions to Reduce Risks. Complete on admission, at change of condition, transfer to a new unit, and after a fall.

Variables		Score	Admission Date	Review Date	Review Date
History of Falling	No Yes	0 25			
Secondary Diagnosis	No Yes	0 15			
Ambulatory Aid	None/bedrest/nurse assist	0			
	Crutches/cane/walker	15			
	Furniture	30			
IV or IV access	No Yes	0 20			
Gait	Normal/bedrest/wheelchair	0			
	Weak	10			
	Impaired	20			
Mental Status	Knows own limits	0			
	Overestimates or forgets limits	15			
		Total			
		Signature & Status			

To obtain the Morse Fall Score add the score from each category.

Morse Fall Score	
High Risk	45 and higher
Moderate Risk	25-44
Low Risk	0-24

Note: Complete checklist for resident assessed based on level of risk.

Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.









Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Fire Safety

- Know evacuation plan
- Protect patients in immediate risk if able
- Report location
- Close patient and fire doors
- RACE and PASS



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Electrical Hazards

- Electrical equipment in the hospital must be well maintained
- Clinical engineering departments inspect equipment
- Nurses should also inspect prior to use
- Report and remove any equipment that does not work or is broken



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Tornado Safety

- Assist patients and visitors to safe and secure location
- Do not use elevators unless absolutely necessary
- Close all patient doors and draw curtains
- If client unable to leave bed, move bed as far from windows as possible
  - Cover client with blankets
- Draw all shades
- Encourage clients to wear shoes



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Think about worst case scenario if tornado comes through and hospital is evacuated after. Get socks and shoes on clients. Check on clients when safe to do so. If able, put client and any visitors in secure inner room such as a bathroom in the patient room.

Active Shooter

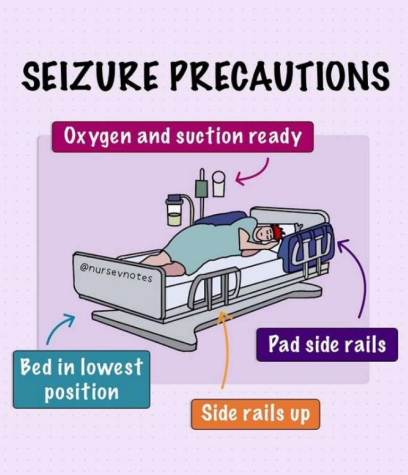
- Run
  - Evacuate if clear path of exit
  - Leave belongings behind
  - Instruct others to follow but do not wait
  - Prevent others from entering building
- Hide
  - Stay out of shooter's sight
  - Find a protected area
  - Lock and barricade doors
  - Silence phone
  - Remain calm and quiet
- Fight



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Seizure Precautions

- If a seizure begins:
  - Turn on side
  - Look at clock and note start time
- Room set up
  - Oxygen and suction ready
  - Padded side rails
  - Bed in low, locked position
  - All four side rails up
  - AMBU bag near bedside



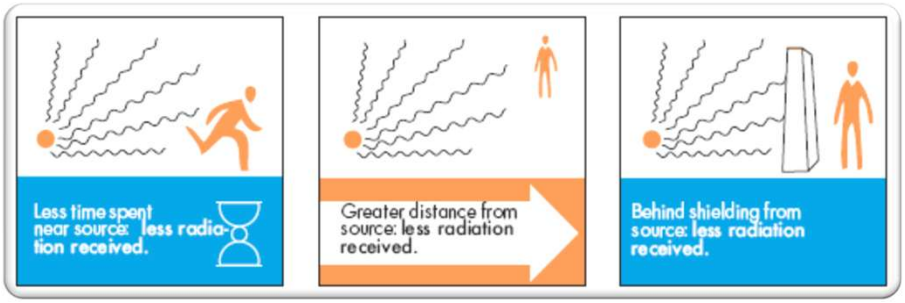
Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Prolonged or repeated seizures indicate status epilepticus, a medical emergency.

Seizure precautions encompass all nursing interventions to protect the patient from traumatic injury, position for adequate ventilation and drainage of oral secretions, and provide privacy and support following the seizure.

Radiation Safety

- Spend as little time near radiation sources
- Stay at least 6ft away
- Utilize shielding or drapes



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Radiation and radioactive materials used in the diagnosis and treatment of patients is a health hazard in health care settings. Protect yourself from radiation.

Restraints

- Restraints can be utilized as a last resort safety intervention
- Clients have the right to be taken care of in the least restrictive environment
- Other interventions should be attempted prior to placing restraints:
  - Offer diversional activities
  - De-escalate
  - Place in a room near the nurse's station
  - Encourage family presence
  - Have a sitter at the bedside
  - Use bed/chair alarms
  - Reorient client
  - Conceal medical devices (ex. IV)
  - Decrease noise and stimulation



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Restraints can be defined in several different ways. Generally speaking, a restraint's purpose is to limit clients' movement and function for their own safety and the safety of other clients and staff. Use of a restraint should be a last resort to achieve client and staff safety. All other interventions should be attempted first, and their effectiveness or lack of effectiveness documented

Restraints cont.

- Indications for the use of restraints include the following situations.
  - The client is exhibiting unnecessary or unwanted movement that is considered unsafe.
  - The client is attempting to remove needed medical items (e.g., feeding tubes, intravenous catheters, indwelling urinary catheters, breathing devices, or drains and bandages).
  - The restraints can provide safety and security for a client undergoing a procedure, both before and during the procedure.
  - The client is aggressive or combative.
  - The client is a danger to self or to others.
  - Other alternatives have been exhausted, and the delay in applying the restraint could potentially cause significant injury or damage.



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Restraints cont.

- Mitten
- Elbow
- Lap belt
- Soft wrist
- Leather wrist
- Chemical restraints



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Restraints cont.

Documentation

- Client behavior that required restraints
- Alternative interventions that failed
- Rationale for type of restraint
- Client behavior/response to restraint
- Criteria for release
- Assessment

Assessment

- Intake and output
- Bathing
- Range of motion
- Toileting

Restraint renewal, documentation, supervision, and assessment policy will vary depending on each hospital



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.

Questions?



Copyright 2022 Assessment Technologies Institute, L.L.C. All rights reserved.